

SECTION 1: Airway & Breathing (Questions 1-20)

1. A 30-year-old patient has a complete airway obstruction and becomes unconscious. The next step is:

- A. Perform abdominal thrusts
- B. Begin CPR starting with chest compressions
- C. Perform finger sweep
- D. Attempt rescue breathing

Answer: B - For an unconscious patient with a complete airway obstruction, begin CPR starting with chest compressions.

2. The correct oxygen flow rate for a nasal cannula is:

- A. 1-6 L/min
- B. 10-15 L/min
- C. 15-20 L/min
- D. 25-30 L/min

Answer: A - Nasal cannula delivers oxygen at 1-6 L/min, providing 24-44% FiO₂.

3. Which of the following is a contraindication for a non-rebreather mask?

- A. Patient with COPD
- B. Patient with chest pain
- C. Patient with a broken nose
- D. Patient who is claustrophobic

Answer: A - NRB masks deliver high-flow oxygen and should be used cautiously in COPD patients who rely on hypoxic drive.

4. A patient presents with wheezing and prolonged expiratory phase. This is most consistent with:

- A. Upper airway obstruction
- B. Lower airway obstruction
- C. Pneumothorax
- D. Pulmonary embolism

Answer: B - Wheezing and prolonged expiration indicate lower airway obstruction (asthma, COPD).

5. The correct technique for suctioning an adult patient is:

- A. Suction while inserting the catheter
- B. Suction for up to 30 seconds
- C. Suction while withdrawing the catheter
- D. Suction continuously for 1 minute

Answer: C - Suction while withdrawing the catheter to prevent mucosal damage. Max 15 seconds per pass.

6. A patient with a tracheostomy requires suctioning. The catheter should be inserted:

- A. As far as it will go
- B. No more than 1 cm past the tracheostomy tube
- C. Only into the tracheostomy tube
- D. Until resistance is met

Answer: C - Insert the catheter only into the tracheostomy tube. Do not insert past the tube.

7. Which of the following is a sign of a partial airway obstruction?

- A. Silent chest
- B. Inability to speak
- C. Weak, ineffective cough
- D. Stridor

Answer: D - Stridor indicates partial airway obstruction. Silent chest indicates complete obstruction.

8. The correct oxygen flow rate for a bag-valve mask is:

- A. 2-4 L/min
- B. 6-8 L/min
- C. 10-15 L/min
- D. 15-25 L/min

Answer: C - BVM requires 10-15 L/min to maintain an adequate seal and deliver high-concentration oxygen.

9. A patient presents with subcutaneous emphysema. This is most commonly associated with:

- A. Asthma
- B. Pneumothorax
- C. Pneumonia

D. CHF

Answer: B - Subcutaneous emphysema (air under the skin) is commonly associated with pneumothorax.

10. The most appropriate treatment for a patient with croup is:

- A. Administer epinephrine
- B. Administer humidified oxygen
- C. Perform endotracheal intubation
- D. Administer albuterol

Answer: B - Croup is treated with humidified oxygen and, if severe, nebulized epinephrine.

11. Which of the following is a sign of epiglottitis?

- A. Barky cough
- B. Tripod position
- C. Wheezing
- D. Chest pain

Answer: B - Epiglottitis presents with tripod position, drooling, and stridor.

12. The correct size endotracheal tube for an adult male is:

- A. 6.0 mm
- B. 7.0 mm
- C. 8.0 mm
- D. 9.0 mm

Answer: C - Adult males typically use 7.5-8.5 mm ETT. 8.0 mm is a common starting size.

13. A patient presents with a flail segment of the chest wall. The most appropriate treatment is:

- A. Apply an occlusive dressing
- B. Stabilize with a bulky dressing
- C. Perform needle decompression
- D. Administer positive pressure ventilation

Answer: B - Stabilize the flail segment with a bulky dressing taped to the chest wall.

14. Which of the following is a complication of endotracheal intubation?

- A. Hypotension
- B. Esophageal intubation
- C. Bradycardia
- D. All of the above

Answer: D - All of the above are complications of endotracheal intubation.

15. The correct confirmation method for endotracheal tube placement is:

- A. Chest rise only
- B. Auscultation of breath sounds only
- C. End-tidal CO₂ detection
- D. All of the above combined

Answer: D - ETT placement should be confirmed by multiple methods: chest rise, auscultation, and EtCO₂.

16. A patient presents with a spontaneous pneumothorax. The most appropriate treatment is:

- A. Needle decompression
- B. Chest seal
- C. High-flow oxygen and transport
- D. Positive pressure ventilation

Answer: C - A stable spontaneous pneumothorax is treated with high-flow oxygen and transport.

17. Which of the following is a sign of respiratory arrest?

- A. Tachypnea
- B. Apnea
- C. Wheezing
- D. Coughing

Answer: B - Respiratory arrest is the absence of breathing (apnea).

18. The correct ventilation rate for a child with a pulse but not breathing is:

- A. 1 breath every 2-3 seconds
- B. 1 breath every 3-5 seconds
- C. 1 breath every 5-6 seconds
- D. 1 breath every 6-8 seconds

Answer: B - For a child with a pulse but not breathing, ventilate at 1 breath every 3-5 seconds.

19. A patient presents with a foreign body airway obstruction. The patient is conscious and coughing forcefully. The most appropriate treatment is:

- A. Perform abdominal thrusts
- B. Perform back blows
- C. Encourage continued coughing
- D. Perform finger sweep

Answer: C - If the patient can cough, encourage coughing. Do not intervene unless the obstruction becomes complete.

20. Which of the following is the most common cause of lower airway obstruction in children?

- A. Epiglottitis
- B. Croup
- C. Asthma
- D. Foreign body aspiration

Answer: D - Foreign body aspiration is the most common cause of lower airway obstruction in children.

NREMT PRACTICE EXAM 2

Cardiovascular (Questions 21-40)

SECTION 2: Cardiovascular Emergencies (Questions 21-40)

21. A patient presents with chest pain and ST depression in leads V1-V4. This indicates:

- A. Anterior wall MI
- B. Posterior wall MI
- C. Inferior wall MI
- D. Lateral wall MI

Answer: B - ST depression in V1-V4 may indicate posterior wall MI (reciprocal changes).

22. The correct treatment for a patient with unstable tachycardia and a pulse is:

- A. Vagal maneuvers
- B. Synchronized cardioversion
- C. Adenosine
- D. Atropine

Answer: B - Unstable tachycardia with a pulse requires synchronized cardioversion.

23. A patient presents with a heart rate of 220 bpm, irregular rhythm, and a wide QRS complex. This is most consistent with:

- A. SVT
- B. Atrial fibrillation
- C. Ventricular tachycardia
- D. Sinus tachycardia

Answer: C - VTach presents with a wide complex tachycardia, typically 100-250 bpm.

24. Which of the following is a sign of left-sided heart failure?

- A. JVD
- B. Peripheral edema
- C. Pulmonary edema
- D. Hepatomegaly

Answer: C - Left-sided heart failure causes pulmonary edema due to backup of blood into the lungs.

25. The correct treatment for a patient with right-sided heart failure is:

- A. Administer diuretics
- B. Administer nitroglycerin
- C. Position the patient supine
- D. Administer high-flow oxygen only

Answer: A - Diuretics are the primary treatment for right-sided heart failure to reduce fluid overload.

26. A patient presents with a blood pressure of 200/110 mmHg and chest pain. The most appropriate treatment is:

- A. Administer nitroglycerin
- B. Administer morphine
- C. Administer aspirin and transport
- D. Administer labetalol

Answer: C - Administer aspirin and transport. Nitroglycerin is contraindicated if SBP < 90 or if PDE5 inhibitors were used.

27. Which of the following is a sign of aortic dissection?

- A. Equal blood pressure in both arms
- B. Tearing chest pain radiating to the back
- C. Bilateral crackles
- D. JVD

Answer: B - Aortic dissection presents with tearing chest pain radiating to the back and unequal blood pressures.

28. The correct treatment for a patient with symptomatic bradycardia and hypotension is:

- A. Atropine 0.5mg IV
- B. Transcutaneous pacing
- C. Dopamine infusion
- D. Epinephrine infusion

Answer: A - Atropine 0.5mg IV is the first-line treatment for symptomatic bradycardia.

29. A patient presents with a heart rate of 150 bpm and a regular narrow complex rhythm. The most appropriate treatment is:

- A. Adenosine 6mg IV
- B. Vagal maneuvers
- C. Amiodarone 150mg IV

D. Lidocaine 1mg/kg IV

Answer: B - Vagal maneuvers are first-line for stable SVT.

30. Which of the following is a contraindication for fibrinolytic therapy in stroke?

A. Age > 80

B. Recent surgery

C. Hypertension

D. Diabetes

Answer: B - Recent surgery is a contraindication for fibrinolytic therapy due to bleeding risk.

NREMT PRACTICE EXAM 2

Medical Emergencies (Questions 41-50)

SECTION 3: Medical Emergencies (Questions 41-50)

41. A patient presents with a blood glucose of 30 mg/dL and is unresponsive. The treatment is:

- A. Administer oral glucose
- B. Administer D50 IV
- C. Administer glucagon IM
- D. Start an IV

Answer: B - Unresponsive hypoglycemic patients require IV D50 or IM glucagon.

42. Which of the following is a sign of opioid overdose?

- A. Tachycardia
- B. Mydriasis
- C. Bradypnea
- D. Hypertension

Answer: C - Opioid overdose causes bradypnea, miosis, and decreased level of consciousness.

43. A patient presents with a seizure. The most important initial action is:

- A. Restrain the patient
- B. Insert an oral airway
- C. Protect the patient from injury
- D. Administer lorazepam

Answer: C - Protect the patient from injury first. Do not restrain or insert objects in the mouth.

44. Which of the following is a sign of increased ICP?

- A. Bradycardia
- B. Hypotension
- C. Tachypnea
- D. Mydriasis

Answer: A - Cushing's triad (hypertension, bradycardia, irregular respirations) indicates increased ICP.

45. A patient presents with severe abdominal pain and rebound tenderness. This is most consistent with:

- A. Appendicitis
- B. Peritonitis
- C. Cholecystitis
- D. Pancreatitis

Answer: B - Rebound tenderness indicates peritonitis, a surgical emergency.

NREMT PRACTICE EXAM 2

Trauma (Questions 51-60)

SECTION 4: Trauma (Questions 51-60)

51. A patient presents with a gunshot wound to the chest. The most appropriate treatment is:

- A. Apply a chest seal
- B. Apply direct pressure
- C. Perform needle decompression
- D. Administer high-flow oxygen

Answer: A - Apply a chest seal to a penetrating chest wound to prevent tension pneumothorax.

52. Which of the following is a sign of decompensated shock?

- A. Tachycardia
- B. Hypotension
- C. Anxiety
- D. Diaphoresis

Answer: B - Hypotension indicates decompensated shock. Tachycardia and anxiety are early signs.

53. A patient presents with a traumatic amputation. The amputated part should be:

- A. Placed directly on ice
- B. Wrapped in moist gauze and placed in a plastic bag on ice
- C. Placed in warm water
- D. Discarded

Answer: B - Wrap in moist gauze, place in a plastic bag, and place on ice. Do not place directly on ice.

NREMT PRACTICE EXAM 2

OB/Pediatrics (Questions 61-70)

SECTION 5: OB/Pediatrics (Questions 61-70)

61. A newborn has an APGAR score of 3 at 1 minute. The most appropriate action is:

- A. Routine care
- B. Stimulation only
- C. Positive pressure ventilation
- D. Chest compressions

Answer: C - An APGAR of 3 indicates the need for positive pressure ventilation and possible intubation.

62. Which of the following is a sign of respiratory distress in a newborn?

- A. Bradycardia
- B. Grunting
- C. Hypotension
- D. Cyanosis only

Answer: B - Grunting is a sign of respiratory distress in newborns.

NREMT PRACTICE EXAM 2

Operations (Questions 71-80)

SECTION 6: Operations (Questions 71-80)

71. The correct order of the scene size-up is:

- A. Scene safety, mechanism of injury, number of patients, additional resources
- B. Number of patients, scene safety, mechanism of injury, additional resources
- C. Mechanism of injury, scene safety, number of patients, additional resources
- D. Additional resources, scene safety, mechanism of injury, number of patients

Answer: A - Scene safety is always the first step in the scene size-up.

72. Which of the following is a sign of a patient who is a danger to themselves?

- A. Calm demeanor
- B. Suicidal ideation
- C. Cooperative behavior
- D. Normal speech

Answer: B - Suicidal ideation indicates a patient may be a danger to themselves.

DISCLAIMER: These practice questions are for educational purposes only. They are not official NREMT exam questions. Always verify answers against current NREMT guidelines and your course materials.